She Is Modest. He Is Focused. She Is Easy:

How South Asian Families Learn to Story Autism Away

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A mother sits next to her seven-year-old son, smoothing her dupatta over her knees. The clinician across from her uses words like “deficit in social reciprocity” and “restricted repertoire of interests". She nods. She signs. She goes home and cries in the bathroom, so her mother-in-law doesn’t hear (Rashid 2026). This is the opening scene of Khadija Rashid’s 2026 Substack essay “When the Word Doesn’t Exist: Autism, Izzat, and the Fight to Be Seen in Brown Families", published on *Brown History*. This is not hypothetical. This is Tuesday. Rashid, a Pakistani-American registered behaviour technician working with autistic children in New York City, tells a story about what happens when disability arrives in a South Asian household without a word to name "it". This essay argues that Rashid’s narrative is a critical disability story that exposes how dominant framings of autism – as individual deficit, as cultural shame, as a problem requiring Western professional intervention – converge to produce compounding structural failures for Brown families navigating disability. Drawing on Mike Oliver’s critique of the individual model of disability, Alfredo J. Artiles’s analysis of the racialisation of disability, Kelly Fritsch’s theorisation of neoliberal debility and capacity, and Theresa Man Ling Lee’s intersectional account of disability, race, and immigration, this essay demonstrates that Rashid’s story is not about cultural misunderstanding. It is about the systematic production of exclusion at the intersection of ableism, racism, and immigration precarity – and the real material consequences this carries for South Asian families navigating disability.

Naming and Not Naming: The Personal Tragedy Story in South Asian Households

Before the clinician's words, before the intake forms, there is a longer story happening inside the household. A child who does not make eye contact is modest, a "virtue". A child who melts down at loud gatherings is sensitive, like her Nani (maternal grandmother). A child who lines up his cars in perfect rows instead of playing with cousins is focused – maybe he'll be an engineer one day. Rashid documents how South Asian families construct these explanations not out of ignorance but out of love, out of cultural frameworks that have genuinely served communities for generations, and out of a rational fear of what happens when you stop explaining and start naming, because naming has consequences.

This is what Mike Oliver calls the personal tragedy theory of disability, but here it operates in reverse. The individual model of disability frames impairment as 'some terrible chance event which occurs at random to unfortunate individuals' (Oliver 1990) – a tragedy to be diagnosed, treated, and fixed. In South Asian households, Rashid shows us another kind of story: one that refuses naming altogether, absorbing the child's difference into existing explanatory frameworks rather than handing it to a clinical apparatus. The phrases 'boys speak late' and 'she's just shy' are not denial. They are rational responses to a real threat – what happens to a family's honour when disability enters the public record?

The individual model also locates 'the problem of disability within the individual' and sees it as stemming from 'functional limitations or psychological losses' (Oliver 1990). The waiting room enacts this model completely: the mother signs forms that ratify a professional determination that her son is the site of the problem. But Rashid's essay troubles Oliver's framework by showing that the alternative story South Asian families tell – the car-lining child

who will be an engineer, the quiet girl who is so well-behaved – is *also* an individualising story. It still locates the problem inside the family's private management of the child, rather than in a society that has provided no culturally accessible pathway to diagnosis and support. Both the clinical story and the cultural story leave structural failure unaddressed. This is the trap Rashid's essay names.

The Weight of Izzat: Race, Immigration, and the Compounding of Exclusion

When a family does reach the clinic, they do not simply enter a medical system. They enter a racialised and immigration-stratified one. Alfredo J. Artiles argues that disability diagnoses for racial minorities create a 'double bind' that compounds the structural disadvantages each group has already endured (Artiles 2013). For South Asian immigrant families, this double bind is acute: to seek a diagnosis is to engage with state institutions that many families have good reason to distrust, whose intake forms are written at reading levels that defeat even fluent English speakers, whose professionals may read culturally grounded deference as a clinical deficit, and whose systems presuppose cultural capital that immigrant families are unlikely to possess.

The depth of this bind becomes visible through documented Canadian policy. In 2016, Felipe Montoya, a Costa Rican professor at York University, was denied permanent residency in Canada because his thirteen-year-old son has Down syndrome. Immigration officials determined that Nico constituted an 'excessive demand on social services', calculating his worth against a per capita cost threshold of $6,387 (CBC News 2016). The state had quantified a disabled child's right to belong. Montoya responded: 'I think the disability community doesn't deserve compassionate and humanitarian considerations, but rather justice and means of inclusion into

society' (CBC News 2016). This is not an isolated incident. South Korean and American families have faced equivalent deportation threats over autistic children in Canada. The logic is consistent: disability, when attached to immigration status, becomes grounds for removal.

Theresa Man Ling Lee's account of her experience as a Han-Chinese woman with a speech disability navigating workplace accommodation in Canada shows how this compounding works at the everyday level. Lee describes how her speech impairment was persistently read through the lens of her race: colleagues and strangers attributed her slurred speech to accent rather than neurological impairment, collapsing disability into ethnicity in ways that served neither her as a disabled person nor as a racialised one (Lee 2005). 'The interesting dynamic here,' she writes, 'is that my race is visible but my disability is not' (Lee 2005). For South Asian families, this dynamic is reversed and intensified: disability, when disclosed, collapses into deficit narratives already primed by racial stereotyping, making the clinical encounter hostile from the outset.

Worthy or Disposable: Neoliberal Debility and the Limits of Access

The structural failures Rashid documents are not accidental. They are the predictable output of a system that, as Kelly Fritsch argues, does not treat disability as a uniformly oppressed identity but as a site of differential inclusion governed by neoliberal logics of capacity and debility. Fritsch, drawing on Jasbir Puar, argues that under neoliberal biocapitalism all bodies are evaluated according to their productive capacity and that disability is not simply excluded but differentially included: capacitated bodies are welcomed into the labour market and serviced by the state; debilitated bodies – those deemed unproductive, too costly, or insufficiently mobile – are left for what Lauren Berlant terms 'slow death' (Fritsch 2015).

This framework clarifies what Rashid is actually describing. The eighteen-month ABA therapy waitlist, the four-thousand-dollar monthly private fee, the IEP meeting conducted without an interpreter – these are not gaps in an otherwise functional system. They are the system functioning as designed, sorting families by their capacity to pay, advocate in English, and navigate bureaucratic complexity. South Asian immigrant families, already disadvantaged by language, cultural capital, and immigration precarity, are positioned at the debilitated end of this sorting. As Fritsch notes, 'disabled bodies that are profitable, that can be marketed to, can be enhanced, or incorporated into the labour force are debilitated bodies that neoliberalism deems worthy' (Fritsch 2015). A seven-year-old Brown child from an immigrant family who cannot access early intervention because his family cannot afford it and cannot wait eighteen months is not a policy failure. He is a policy *outcome*.

Rashid does not use the language of debility and capacity, but she describes its effects. For families who cannot afford the private system and cannot survive the public waitlist, autism services become a luxury indexed to class. The families Rashid describes are not failing to access care because they do not want it. They are being excluded by a system that was not designed to hold them.

The Gaze That Misses: Brown Girls, Double Invisibility, and Dis-identification

If the average autistic Brown child is underdiagnosed, the autistic Brown girl is practically invisible. Rashid's account of this double invisibility is the most analytically piercing section of her essay. Autistic girls are missed by the clinical gaze – which has historically centred on male presentations of autism – because they mask well. They are missed by the cultural gaze

because the behaviours that constitute masking – quietness, compliance, and social deference – are read in South Asian household frameworks as ideal femininity. As Rashid documents, autistic girls are often praised for being ‘quiet’ or ‘easy’ when they are, in fact, exhausted from the performance. She is the ideal South Asian girl. She is also entirely unseen.

Lee’s concept of dis-identification is useful here. Lee argues that the logic of accommodation works through dis-identification: the goal is not to celebrate a positive group identity but to allow an individual to no longer be defined by their impairment in contexts where it creates a handicap (Lee 2005). For the autistic Brown girl, dis-identification is what the system denies her. She is never identified in the first place. Without a diagnosis, there is nothing to accommodate. Without accommodation, her masking intensifies. Without anyone naming what she is doing, her exhaustion is read as a virtue. The system does not fail to disidentify her. It never identifies her at all.

This double erasure – by both clinical and cultural frameworks at once – is what Artiles would call structural intersectionality: her experience at the intersection of gender, disability, race, and culture is qualitatively different from, not just worse than, the experience of any single axis alone (Artiles 2013). No single-axis intervention can reach her. She requires an analysis that holds all of these axes at the same time and a system that is built to do the same.

Conclusion: The Maze Does Not Disappear

At the end of Rashid’s essay, there is a question that never quite leaves: ‘What happens to him when we are not here?’ It sits underneath every other question – underneath the IEP meeting, the waitlist, the izzat calculation, the Brown girl praised into invisibility. It names what

is really at stake when disability stories in South Asian communities remain unspoken, when the word does not exist, when communities have not yet built the language or the infrastructure to hold these families with the visibility they need and deserve.

Read through Oliver's critique of the individual model, Artiles's account of racialised double binds, Fritsch's analysis of neoliberal debility, and Lee's account of dis-identification; Rashid's story becomes legible as a disability justice text that refuses both the personal tragedy narrative and the tidy social model correction. The Montoya case in Canada makes explicit what Rashid's essay documents in the clinical and familial register: the state, no less than the clinic or the community, reads disabled Brown bodies through a framework of excessive demand rather than inherent belonging. As Montoya himself said, this community does not deserve compassionate consideration. It deserves justice (CBC News 2016).

The structural change Rashid calls for – in how clinicians are trained, how schools communicate rights, how community institutions show up, and how we as a culture decide to talk about difference – is not modest. It is the minimum. Awareness is not enough. Representation is not enough. The question that never quite leaves deserves an answer that meets it honestly: not managed concessions to those who are different, but the kind of full structural transformation that makes it possible for Brown families navigating disability to exist fully, visibly, and at home in their own communities.

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